Pembrolizumab + carboplatin + fluorouracil

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab alone

Active medicines: Pembrolizumab

Emetogenic risk: minimal

Incomplete clinician/pharmacy review draft. Individualise and authorise before use.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Pembrolizumab can cause inflammation in healthy organs even after chemotherapy has finished. Keep the alert card and report new symptoms promptly. Steroids or hormone replacement may be needed for specific complications; do not start or stop them without the treating team.
- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- No routine prophylactic antiemetic is specified for pembrolizumab alone. Do not carry platinum antiemetics or hydration forward automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00705 v2d · pSee regimen treatment, tests and supportive-care sections · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-07 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required